CO2060 – Software Systems Design Project

Meeting Notes – User / Stakeholder Meeting

HOPEDROP – Blood Bank Management System

Group G50 DevDynamos

Meeting #U1

Date: Sunday, 01 February 2026

Date:	Saturday, 01 February 2026
Location:	Online (Discode)
Time:	10:00 PM – 11:00 PM
Facilitator:	K.D.Ashen
Attendees:	K.D.Ashen, G.K.G.Sandeepa, R.M.S.S.Kumara, A.W.H.Panchani, Stakeholder

1. Meeting Agenda

- Introduce HOPEDROP project and its objectives
- Validate functional requirements with real-world medical perspective
- Identify pain points in current blood bank management process
- Gather feedback on proposed system features

2. Key findings from stakeholder

- Current process relies on phone calls between hospitals to check blood availability; highly inefficient during emergencies
- Doctors are unaware of blood stock in hospitals even 5 km away during critical cases
- Manual donor eligibility checks cause delays; an automated engine would significantly speed up the process
- Real-time stock updates are critical; a 3-second or less latency requirement was validated as realistic and necessary

- The Disaster Mode concept was positively received; simplifying the UI for bulk requests during mass casualty events is a high-value feature
- Doctor expressed concern about data accuracy: if a lab technician enters wrong blood type, it could be fatal; barcode scanning was suggested as a mitigation
- Donor feedback (knowing if their blood was used) was identified as a motivator that could improve donor retention
- Inter-hospital blood exchange feature was confirmed as essential and legally feasible between government hospitals

3. Requirements validated

- FR1 : Lifecycle Tracking: Confirmed as critical
- FR2 : FIFO Policy: Agreed necessary to reduce wastage
- FR3 : Predictive Stock-out Alerts: 48-hour threshold validated as appropriate
- FR5 : Disaster Mode: Positively received; bulk request simplification agreed
- FR6 : Donor Eligibility Engine: Confirmed as necessary for safety
- NFR3 : Performance (10,000 concurrent users): Agreed as reasonable for national system
- NFR5 : Real-Time Latency (3 seconds): Validated as acceptable

4. New requirements identified

- Inter-hospital blood exchange (authorized staff can view and request from nearby hospitals)
- Donor feedback loop; notify donor when their donated blood is issued to a patient

5. Out of scope items confirmed

- Physical logistics and transportation of blood packets
- Direct integration with automated blood testing hardware
- Financial transaction processing

6. Next Steps

- Update SRS document with new requirements identified during interview
- Add Inter-Hospital Exchange as a functional requirement
- Share updated SRS with doctor for review if possible

Signatures

Tech Lead

K.D.Ashen

Signature

Stakeholder

G.K.G.Sandeepa

Signature